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Kenneth A. Rethmeier
is Founder and President of
The Rethmeier Group, LLC.
He holds a doctor of public
health from the University of
North Carolina and has had a
highly decorated career in the
US Navy Medical Service
Corps. He has built up a
record of excellence in
executive coaching and
consulting with individuals,
teams and healthcare
organisations internationally.
Building on his extensive
business experience, Dr
Rethmeier has developed
techniques to help
organisations transform ideas
into vision and direction,
learning into capacity and
capability, and experience into
action.

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Kenneth A. Rethmeier
The Rethmeier Group, LLC
421 Orchard Park Drive
Bermuda Run
North Carolina 27006
USA

E-mail: krethmeier@
therethmeiergroup.com

Innovation for healthcare reform: Creating opportunities to explore, expand and excel

Kenneth A. Rethmeier

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Abstract

In the face of an uncertain future, ongoing political turbulence, the global economic downturn, and the frequent changes to the direction of healthcare reform in many countries, healthcare leaders face daunting challenges. As a consequence, healthcare managers at all levels need to be willing to discard the thinking and behaviours of their past and to learn new ways to embrace the opportunities before them. In short, meaningful reform in healthcare depends on innovation. Moreover, innovation is a core and critical leadership competency for sustained superior performance. This paper offers new skills and tools based on the principles of creativity and innovation. Using these tools, leaders can: have more productive conversations with their leadership teams; improve their ability to facilitate management sessions to deal more effectively with critical business issues; produce meaningful action and results to overcome the obstacles of healthcare reform; and have a greater appreciation that the premium on creativity is rising.

Introduction

In the face of an uncertain future, ongoing political turbulence, the global economic downturn, and the frequent changes to the direction of healthcare reform in many countries, healthcare leaders face daunting challenges.^{1,2} As a consequence, it is difficult to craft a vision for the future and the requisite strategic initiatives to ensure success. Moreover, leaders commonly encounter resistance from their own staff when attempting to make the changes they know will make a difference. Healthcare leaders at all levels must be willing to discard the thinking and behaviours of their past and to learn new ways to embrace the opportunities before them. Success in the future will require a combination of creative and critical thinking skills to help organisations 'rethink their thinking'.

In short, meaningful healthcare reform depends on innovation. This paper will provide healthcare market leaders with a series of tools with which to engage their leadership teams and staff in an effective dialogue, leading to innovation in the healthcare reform process. The paper will not

focus on innovation in terms of advances in technology, pharmacology, diagnostic procedures, nor the process of providing healthcare services. Rather, it will provide leaders and managers with the tools to become more effective in guiding and moving their healthcare organisations forward to confront the challenges of healthcare reform. The tools contained in this paper will offer leaders new ways to think about and craft their own efforts to promote change in healthcare reform among the community, providers and patients they serve. The tools presented in this paper have equal application. The overarching goal is to offer healthcare organisations an opportunity to enhance their leadership and management skills and systems with the capability, capacity and confidence to ensure sustained superior performance in this period of intense transition.

Context and assumptions

Within Europe in particular, there are a number of very different health systems, with a variety of historical backgrounds, yet there are also a number of common challenges for healthcare reform. The following list of assumptions anchors the case for creativity and innovation:

- Healthcare by its very nature manifests a country's core social values.
- Albeit at different rates of change, demographic shifts across Europe are creating greater strains on healthcare systems. Old-age dependency ratios signal an eroding workforce, leaving countries with ever-increasing gaps in funding. For example, one official from Austria announced at the EHMA conference in June 2009 that, in 2010, the country will face its first national deficit in funding for healthcare in over 45 years.
- Healthcare costs continue to rise in the absence of adequate constraints. There are never sufficient funds to satisfy the appetite and demands for healthcare services. The only other options are to cut quality and/or services to meet these demands.
- Policy makers and government leaders set the tone and parameters of the environment for opportunities for change.
- Especially in Central and Eastern Europe, the greater degree of paternalism in healthcare systems constrains creativity and ideation, meaning fewer opportunities for meaningful innovative reform. Paternalism equates to control by the central government. Consequently, as governmental control increases, healthcare leaders look towards the central government for answers to problems rather than exercising their own initiative. This is a natural outcome that stifles creativity and innovation.
- Healthcare consumers are smarter, more informed, and more demanding as regards their own role in managing their health.
- Innovation is a critical leadership competency for now and the future and, if organisationally institutionalised, can be a key driver of sustained superior performance.
- Now is the time to explore options and focus on innovative solutions to meet and create a future for success.

Operating definitions

For the purposes of this paper and for clarity of terminology, 'innovation', 'reform' and 'creativity' will be defined as per the *Meriam-Webster Dictionary*:

- *Innovation*: the introduction of something new, such as an idea, method or device; novelty.
- *Reform*: to put or change into an improved form or condition; the removal of faults or abuses.
- *Creativity*: to produce through imaginative skill.

Figure 1 illustrates the context and model used to frame this work towards building a culture of creativity and innovation.

Here one can see that innovation comes from an interactive process of exploring and expanding one's thinking about ideas, relationships, processes and systems to produce new ways of achieving excellence in performance. In the context of healthcare reform, it can mean the healthcare organisation finds new ways to cope or compensate for the challenges of governmental mandates for reform. In other cases, the model supports opportunities for healthcare leaders to craft, develop and implement new methods, processes and systems designed to build capacity and capability to meet healthcare needs where funding is grossly constrained. One such example is the work being done at the Rehabilitation Hospital in Baile-Felix, Romania, where the hospital director, Lazlo Ritli, is transforming not only the physical facility, but changing the way healthcare services are provided. Hospital staff see how they can make a difference in the health and wellbeing of the patients and community they serve. Here is a leader whose professional will and personal humility truly characterise a vision-led organisation. Dr Ritli sets a high standard for others to emulate and his commitment to excellence is working and reaping superior benefits.

Areas for exploration

In the effort to create meaningful healthcare reform, there is a broad spectrum of fruitful areas for exploration. They include, but are not limited to: coverage, services in the programme, cost sharing, management of the benefit package, processes of care and outcomes, structure of the 'plan', expenditure control, payment structure and guidelines, primary care physicians, degrees of system integration, and the spectrum of providers, to name just a few.³



Figure 1: A model for organisational excellence

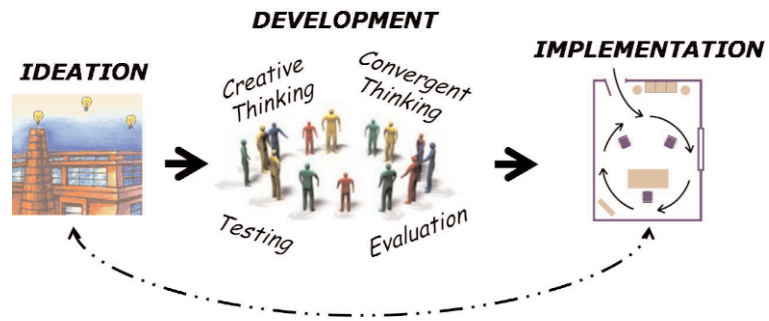


Figure 2: A model of innovation

Model of innovation for reference

The literature on innovation is replete with a full range of types of models, numbers of steps in the process and whether they are driven by internal or external forces, open or closed systems, and the types and configurations of groups of individuals that produce innovations. For the most part, as well, innovation is generally directed at products, technology and processes (such as clinical pathways and/or new diagnostic and treatment protocols in healthcare). The present context is all about leadership and management with the assumption that leadership and management decisions may have significant clinical implications. The model illustrated in Figure 2 consists of three key parts: ideation, development and implementation.

Perspectives on rethinking healthcare reform

For this paper, healthcare reform will be viewed from the interconnected perspectives of time and space. Most of the healthcare reform efforts in Europe consist of cost-containment while maintaining quality of care. In countries like England and Wales, there are ongoing initiatives to decentralise selected funding and management, such as contracting for primary care services while linking performance outcomes to national health status improvement indicators as an incentive. In Germany, the pharmaceutical industry is coming under greater scrutiny through the assessment of the cost benefits of prescriptions to determine reimbursement levels. In some areas, such as Eastern Europe, reform efforts extend into building accountable systems, a greater emphasis on disease prevention and new models of delivering primary care services, and work towards enhancing leadership capabilities to guide sound health system reform.

In this context, the following guide has been developed to evaluate the potential effectiveness of any healthcare reform effort. Time, as shown in Figure 3, will be defined by where the reform initiative fits in terms of primary, secondary and tertiary prevention, with the goal to target the earliest time period (primary prevention) as the one with the best outcome in terms of cost and quality of care.⁴

Secondly, as presented in Figure 4, healthcare reform initiatives designed to change models of thinking have the greatest potential to



Figure 3: Time perspective of prevention

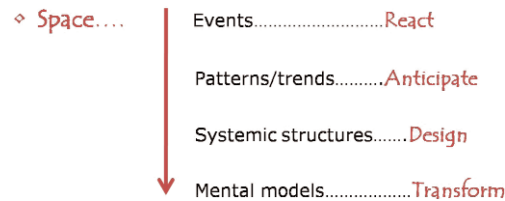


Figure 4: Space: A perspective of systems thinking

transform healthcare.⁵ Conversely, if one addresses only the events level, the best that can be achieved is merely reacting to the need of the system. This is analogous to a physician who treats only the symptom of disease without probing for a deeper understanding and protocol to address the root cause of the disease. The deeper one goes, the better the potential outcome.

In summary, healthcare reform can be represented in a variety of perspectives and can originate with the proactive efforts of individual healthcare organisations and/or actions mandated by a central governmental agency. In either case, organisational action starts with leaders having the ability and tools to engage senior leadership teams in an effective dialogue and solution-finding process. Below, tools are offered to facilitate a productive dialogue for effective outcomes.

Tools for innovation in healthcare reform

Six management tools are offered to assist leaders identify innovative solutions to the challenges of healthcare reform: vision vs current reality; can/cannot control; brainstorming as a technique for generating ideas; 'blue slips' for capturing thinking; the iceberg for systems thinking; and the time-space assessment matrix. The paper will give a brief overview of each management tool and describe 'practice fields' where it can be best applied. These tools offer leaders more effective ways to engage in a productive dialogue to identify and pursue opportunities for innovation in healthcare reform.

Tool 1: Vision vs current reality

Visioning is critical in facilitating a smooth transition in periods of healthcare reform and in the development of strategic initiatives best suited to set the direction for the organisation. A vision is best defined as a picture of a future desired state for the healthcare organisation. It describes what the leader and the leadership team desire for the organisation.

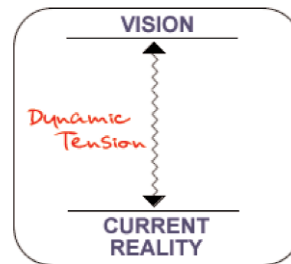


Figure 5: Vision versus current reality
Source: <http://www.partnershipsforolderadults.org>

In some respects, visions are both descriptive and prescriptive because they help the organisation organise itself to think and act differently.

Outlining a future towards which everyone wants to work requires time spent considering the ‘current reality’ — that is, an honest, frank and open dialogue about the present situation. It is often referred to as ‘confronting the brutal facts’. To build a strategic plan with sufficient substance to change the future, communities need a clear sense of where they are now. Figure 5 illustrates the relationship between a vision and current reality. The vision at the top illustrates how the vision will ‘pull’ the organisation towards it. However, visions must confront the ‘current reality’ of the organisation, which is all about where the organisation is today.

One outcome of this exercise is the discovery of ‘unspeakables’. Unspeakables are those things or topics which organisations find it difficult to discuss in public, even though they exist and govern much of the behaviour within the organisation. They typically influence such activities as the interactions that take place in meetings. For example, in organisations where the leader dominates and controls the dialogue of meetings, staff members learn, often through painful experience, that remaining silent during controversial or challenging conversations is better than speaking one’s mind. In such circumstances, when the leader says they want input on a decision, they actually mean the opposite, and will make their decision without it. In such situations, speaking up may leave the staff member vulnerable to public criticism or being ignored by the leader. The result is that decision meetings do not achieve what they could, but no one is willing to speak up out of fear. This is an unspeakable.

Unspeakables exist in every organisation and take on many forms. By themselves, they are not necessarily bad, just dysfunctional. However, when unspeakables become unspeakable, that is, when no one will talk about them, they become pathological. As a consequence, they work like a cancer in the organisation, creating problems and disrupting sound management. Using this tool effectively can help to expose the unspeakable, allowing a more productive conversation about how they adversely affect the organisation and what the leadership team can do to change the culture for improvement.

Current reality is the combination of forces that will help to achieve the vision, in addition to forces that create resistance to the vision (Figure 6). Understanding the differences and how to use the forces supporting the

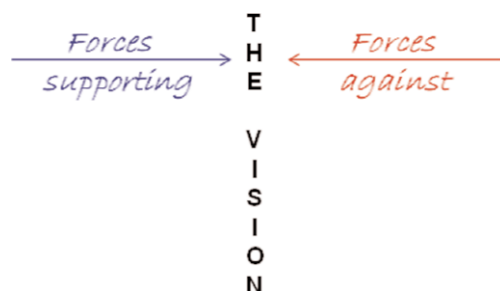


Figure 6: Vision: The force field

vision can support efforts towards achieving it. Meanwhile, having a clear picture of the forces against the vision will set the stage for developing effective initiatives to work around or through them to overcome their negative influence. Unfortunately, most healthcare organisations do not have a clear picture of either and this exercise is most helpful in developing the learning capacity of the organisation.

Practice fields

This tool can be used to promote a productive dialogue to:

- identify and define the organisation's vision for healthcare reform;
- promote understanding of thinking aligned with the vision;
- discover 'today' for the organisation, which means taking time to step back and make an *honest* assessment of the organisations vision in terms of those forces which will help it to achieve its vision juxtaposed with those forces which will be obstacles to achieving that vision;
- brainstorm potential strategies and focal points for innovations to close the gap between the vision and current reality;
- promote awareness, reflection and understanding of the important attitudes and beliefs at the foundation of the organisation's values.

Tool 2: Can/cannot control

When responding to the challenges of healthcare reform, leaders often face situations that make it difficult to differentiate what they can and cannot control (Figure 7). This creates special challenges for efforts to innovate because it is important to know in advance whether an innovation will have the opportunity to succeed. This second tool offers leaders and organisations a simple approach to help differentiate those

	CANNOT CONTROL	CAN CONTROL
PROACTIVE	WASTED TIME	MOVE FORWARD
INACTIVE	LET GO	GIVING UP

Figure 7: The can/cannot control matrix

areas for which there are opportunities to explore and expand creative thinking in search of innovation and those for which there are not.

This tool promotes an effective dialogue about innovations in healthcare reform which lead to high-impact change. The tool illustrates that there are some areas that leaders can control and some they cannot. In each case, leaders and organisations can be either proactive or inactive in their efforts. Between the two parts of the matrix, there are four possible outcomes. Where leaders and organisations can control and be proactive in their efforts, these are areas where it is possible to move forward. For areas where leaders can control but are inactive in their efforts, they are really just squandering opportunities for significant results. Turning toward areas which cannot be controlled, two possibilities result. First, if areas cannot be controlled, yet leaders and organisations feel the need to be proactive, in other words, persist in efforts to exercise control, they are in reality wasting their time and effort, and should, in short, stop. Conversely, if areas cannot be controlled, and leaders are inactive, then it is important just to let those areas go, at least for now.

Practice fields

Some potential benefits from exploring this particular tool in healthcare reform are to:

- clarify and confirm areas for which leaders and organisations do and do not have control — this is critical;
- help set areas for potential brainstorming of innovative solutions;
- further the learning agenda for the leadership team and organisation;
- expose blind spots with both positive and negative influences on the organisation;
- promote critical and creative thinking about what the organisation can do to respond and/or take control of healthcare reform.

Tool 3: Brainstorming

Brainstorming is a technique employed by groups to generate a set of ideas to solve a problem. It is most helpful when groups or teams need to break away from old patterns of thinking, so they can develop new ways of looking at things. It also capitalises on bringing out the diverse experience of the team to enhance the richness and quality of the ideas generated. The more ideas, the greater opportunity to find a better solution.

Practice fields

While there are many techniques and approaches to accomplish an effective brainstorming session, there are some fundamental guidelines to assist the group in generating the greatest number of ideas. They include:

- leaders or organisations need creative thinking to help solve problems, especially when dealing with the challenges of healthcare reform (these challenges need thinking for the future and not just what has worked in the past);
- focus on generating as many ideas as possible, regardless of how wild and different they may be; in fact, encourage extreme thinking — the wilder the better;

- use different techniques to capture the creative thinking process — for example, writing down ideas and/or calling them out and capturing on flip-charts will offer participants different ways to participate;
- focus the participants' attention by establishing clear expectations and goals;
- set a level playing field so that participants feel everyone has an equal contribution to make — leaders need to take off their 'leader hats' during brainstorming sessions;
- do not judge ideas as they are generated — rather, let the group's imagination flow openly — the more ideas the better. Judgment is a form of critical thinking that can hinder the creative process of brainstorming. All ideas are good ideas. Creative thinking can be followed by a structured process of critical thinking to evaluate the usefulness of ideas.

The outcomes from brainstorming sessions will provide a wealth of potential areas for effective innovations for healthcare reform. As groups become more comfortable with brainstorming sessions, the more productive they become. As described below, one method for capturing thinking and ideas from brainstorming sessions is with the use of 'blue slips'.

Tool 4: Blue slips

Blue slips are simply uniformly shaped pieces of light-blue paper used to write down and capture ideas. They are made by cutting a ream of standard-sized paper into eight equal sized pieces. During a brainstorming session, the facilitator asks the group to grab some blue slips and write a key word or letter in the upper left-hand corner, writing no more than one idea on each slip.

During brainstorming sessions, groups are often asked to capture ideas in a series of sequential time-limited sessions, for example, in sets of 90 seconds, 60 seconds, and then in 30 seconds, with the blue slips collected at the end of each set. Using progressively shorter time limits for successive sets actually promotes more creative thinking rather than constraining it. After multiple sessions using this technique, more ideas tend to be produced in the shorter-timed sets. Moreover, groups feel more empowered when the number of ideas generated during each set are counted at the close of a session. Blue slips generated during brainstorming sessions create the starting point for critical thinking to take over and to review the ideas to find those with the greatest promise for implementation.

Practice fields

Here are some thoughts about how blue slips add value to the process of creative thinking for innovative solutions. They:

- capture ideas and produce collective wisdom and new knowledge from which innovations emerge;
- stimulate balanced thinking;
- minimise narrow-mindedness by opening new thinking and creativity;
- can be used to capture participants' thinking about the effectiveness of meetings and/or how to improve them;

- can be kept in one's pocket as one moves around the organisation to capture ideas as they come up.

Keys for their successful use include the following:

- have the session facilitator add the 'keyword' in the upper left-hand corner of each blue slip;
- write only one idea per blue slip;
- generate as many ideas as possible;
- use Post-it notes if there is a need to stick the ideas on a wall for participants to see.

Tool 5: The iceberg

In the words often attributed to Albert Einstein: 'We cannot solve our problems with the same thinking we used when we created them'. In the complex world of healthcare reform, systems thinking, popularised by Peter Senge, helps leaders manage the impact of reform initiatives so their organisation is best positioned for sustained superior performance.⁵ Systems thinking reduces complex issues and challenges into simpler, more understandable terms. Regrettably, many organisational challenges leave leaders without a structured process to analyse and then create innovative solutions. In fact, when critically analysed, most solutions are merely reactions to the symptom's problems and leave the underlying causes unexplored. The iceberg (Figure 8) helps leaders and organisations find innovations that will help them to transform rather than just react to the problems and challenges of healthcare reform.

The iceberg guides one through a series of questions to uncover the underlying systemic forces affecting the critical issue under review. In the case of healthcare reform, recall that to achieve the best results, organisational responses address both time (ie levels of prevention) and space (ie the depth of opportunity to reach truly transformational changes that will produce the most cost-effective outcomes). Two of the goals reported by most European countries for their efforts at healthcare reform are to shift from curative medicine towards preventive medicine and to search for solutions with the highest levels of access to quality medical services. These can only be achieved when leaders realign their thinking and the tools available to them and their leadership teams.

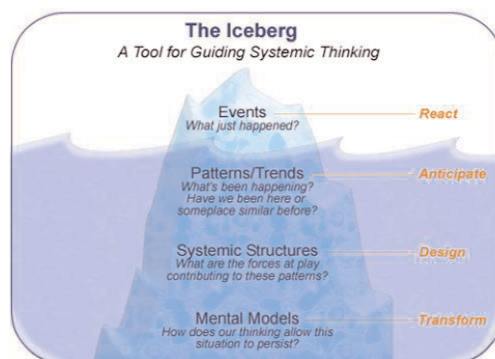


Figure 8: The iceberg — A tool for guiding systemic thinking
Source: <http://www.partnershipsforolderadults.org>

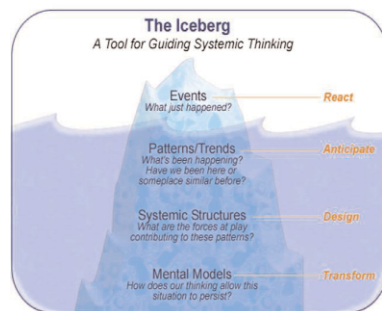


Figure 9: The iceberg — England and Wales

Two examples of the iceberg

The iceberg is useful for uncovering the underlying structure of a healthcare reform initiative which will shape the organisation and its ability to perform optimally in the new environment. In England and Wales, the decentralisation of contracting for primary care services is being used to transform the mental model, moving from curative medicine towards preventive care (Figure 9). This new approach will dramatically shift measurement of the performance of primary care physicians from a 'services' perspective towards improvements in health status.

In Germany, the iceberg helps to explain the systemic dynamics associated with changes to how pharmaceuticals are being reimbursed on a cost-benefit analysis basis versus the former model of 'free pricing', where reimbursement was not tied to any key performance indicator (Figure 10). Prescribed drugs must now meet the test of true clinical cost and quality benefit assessment before they will be properly reimbursed.

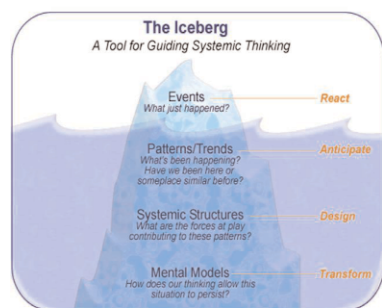


Figure 10: The iceberg — Germany

An example:

England & Wales – "World Class Commissioning"... decentralized contracting for primary care services...what does this mean?

React.....Central to Local Contracts

Anticipate.....Provider Certification

Design.....Curative to Preventative

Transform.....Performance Based on Health Status
 - person-centered $\neq$ patient
 - "consumer" driven
 - MD as facilitator/coordinator
 - improved health status

An example:

Germany... "free pricing" of pharmaceuticals to "best care" assessment...what does this mean?

React.....Cost-Benefit Analysis
 Determines Reimbursement

Anticipate.....Rational Model for
 Management of Future Costs

Design....."Take More Self
 Responsibility"

Transform....."Times of Abundance Are
 Gone...Prioritization"
 - significant implications
 - "unspeakable"...there's an elephant in the room...!
 - how do you prepare & change for success?

Level Of Reform	Levels of Prevention		
	Primary	Secondary	Tertiary
Events			
Trends			
Design			
Transform			

Figure 11: The time-space assessment matrix

Practice fields

Leaders can use the iceberg to help them and their leadership teams in times of dramatic shifts in healthcare reform. It is worth using the iceberg to:

- explore and expand a dialogue regarding current healthcare reform efforts to uncover the thinking behind the initiative;
- examine and identify opportunities for innovations in response to (or pursuit of) reform efforts;
- test how leaders can create systemic change within their organisations as a means to ensure system leadership;
- challenge the mental models, and in this case, the fundamental business model by which the healthcare organisation operates — it may not be the model best suited for future success.

Tool 6: Time-space assessment matrix

The final tool (Figure 11) is designed to help leaders and their leadership teams use the results of their work in creating innovations for healthcare reform to determine where they fit within the context of the time-space concept presented previously. After creating the healthcare reform initiative they believe is best suited for the challenge their organisation faces, the task is to assess its merits carefully based on the time-space perspective and to place it in one of the cells of the matrix. The overall goal, in short, is to create innovations that offer the best solutions to the challenge of healthcare reform and which will move the mental model from curative medicine towards preventive care while simultaneously seeking transformational change.

Practice fields

Here are a few opportunities to help leaders challenge their leadership teams and organisations to seek innovations in healthcare reform:

- scenario planning to test the ability to move thinking and creative innovations down and to the left side of the assessment matrix;
- uncovering current thinking of policy makers or stakeholders in the process of crafting innovations for healthcare reform;

- illustrating for decision makers the value of experimenting with differing business models that emphasise the addition of value to their healthcare organisation's contribution to the health and wellbeing of the communities they serve;
- a foundation for strategy development and crafting a new and innovative organisational vision for the future.

The call for innovative leadership

So what does it take from leaders who want to move their healthcare organisations to the forefront of healthcare reform? What principles and competencies are necessary for transformational change and the challenge of shifting the foundations of systemic structures and mental models that currently govern an organisation's interactions and behaviour? Do leaders submit to the notion that it is better to create one's own future than to have it imposed?

These are the critical questions that the leaders of today's healthcare organisations need to ask themselves, as no one else will — or, rather, it is not someone else's responsibility. This is truly a call for vision-inspired leadership. While research suggests that many leaders desire innovation as part of the DNA of their organisation, most also report a significant gap between the aspiration for innovation and execution. There are three fundamental ways leaders can move organisations towards innovation as a 'this is the way we do things around here' model: setting the vision and strategy using creative thinking and innovation as foundational commitments and values of the organisation; recruiting, selecting and tapping existing organisational talent to build a core of staff dedicated to the notion that creativity and innovation are key to their success; and continuing along the journey of building 'trust' as a critical component of an environment where ideas are valued, people are encouraged to express diverse ideas, and risk taking is rewarded. Now is the time to explore, expand and excel. Leadership is having the courage to act, to do what is right, not to wait and see, not to wait and hope, but, the courage to get involved now.

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